



BARBADOS NATIONAL REGISTRY

Thirty-day case fatality after cardiovascular events

Barbados, 2010-2025

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2025 IN BRIEF

Thirty-day case fatality at a glance

CVD | WOMEN

29.8%

95% CI 25.8-34.1

HEART | WOMEN

37.0%

95% CI 29.4-45.3

STROKE | WOMEN

26.6%

95% CI 22.1-31.8

CVD | MEN

28.6%

95% CI 25.0-32.5

HEART | MEN

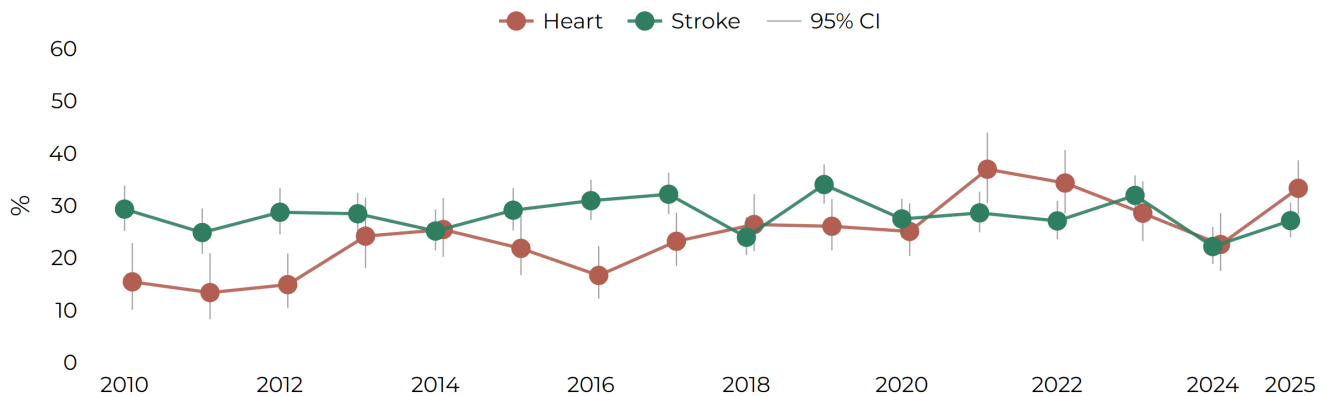
30.6%

95% CI 24.4-37.6

STROKE | MEN

27.6%

95% CI 23.2-32.4



2025 results by sex

Event	Sex	Events	Deaths	Crude % (95% CI)	Adjusted % (95% CI)
CVD	Both	999	291	29.1 (26.4-32.0)	29.5 (26.7-32.4)
CVD	Women	457	136	29.8 (25.8-34.1)	28.2 (24.1-32.3)
CVD	Men	542	155	28.6 (25.0-32.5)	30.7 (26.8-34.7)
Heart	Both	321	107	33.3 (28.4-38.7)	33.2 (28.2-38.1)
Heart	Women	138	51	37.0 (29.4-45.3)	34.3 (26.7-41.9)
Heart	Men	183	56	30.6 (24.4-37.6)	32.2 (25.6-38.8)
Stroke	Both	678	184	27.1 (23.9-30.6)	27.7 (24.4-31.0)
Stroke	Women	319	85	26.6 (22.1-31.8)	25.5 (20.9-30.1)
Stroke	Men	359	99	27.6 (23.2-32.4)	29.9 (25.1-34.6)

WHAT THIS MEANS

In 2025, about three in ten CVD events were followed by death within 30 days. The heart estimate was higher than the stroke estimate, although the uncertainty ranges overlap. Women and men had similar overall crude results. Age adjustment made little difference to the combined-sex estimates, so age mix did not materially change the overall 2025 result.

CVD EVENTS

Thirty-day case fatality after CVD events

Annual results by sex.

CRUDE | WOMEN + MEN | 2025

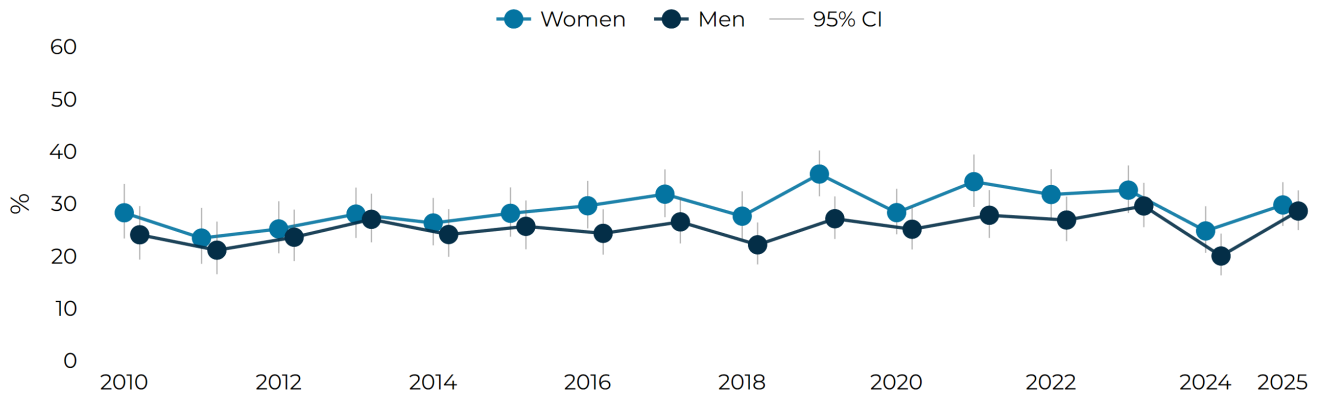
29.1%

95% CI 26.4-32.0

AGE-ADJUSTED | WOMEN + MEN | 2025

29.5%

95% CI 26.7-32.4



Recent results by sex

Year	Sex	Events	Deaths	Crude % (95% CI)	Adjusted % (95% CI)
2021	Women	342	117	34.2 (29.4-39.4)	32.2 (27.4-37.0)
2021	Men	367	102	27.8 (23.5-32.6)	30.0 (25.4-34.5)
2022	Women	381	121	31.8 (27.3-36.6)	30.1 (25.7-34.6)
2022	Men	413	111	26.9 (22.8-31.4)	29.0 (24.8-33.3)
2023	Women	405	132	32.6 (28.2-37.3)	30.4 (26.0-34.8)
2023	Men	443	131	29.6 (25.5-34.0)	31.9 (27.6-36.1)
2024	Women	359	89	24.8 (20.6-29.5)	24.3 (20.0-28.7)
2024	Men	385	77	20.0 (16.3-24.3)	22.7 (18.3-27.1)
2025	Women	457	136	29.8 (25.8-34.1)	28.2 (24.1-32.3)
2025	Men	542	155	28.6 (25.0-32.5)	30.7 (26.8-34.7)

WHAT THIS MEANS

The 2025 crude result was similar for women and men. After age adjustment, the estimate was slightly higher for men, suggesting that differences in age mix affect the sex comparison somewhat; the uncertainty ranges still overlap. The overall 2025 estimate returned to the range seen in 2021-2023. The apparent drop in 2024 should be interpreted cautiously; decisions should consider the multi-year pattern rather than one annual point.

HEART EVENTS

Thirty-day case fatality after Heart events

Annual results by sex.

CRUDE | WOMEN + MEN | 2025

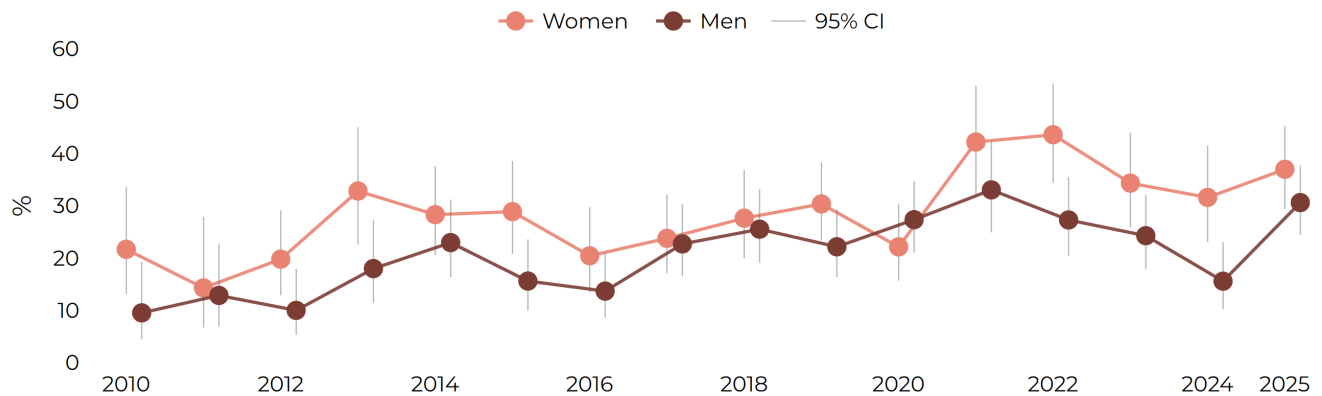
33.3%

95% CI 28.4-38.7

AGE-ADJUSTED | WOMEN + MEN | 2025

33.2%

95% CI 28.2-38.1



Recent results by sex

Year	Sex	Events	Deaths	Crude % (95% CI)	Adjusted % (95% CI)
2021	Women	83	35	42.2 (32.1-52.9)	39.9 (29.8-50.0)
2021	Men	109	36	33.0 (24.9-42.3)	36.1 (27.6-44.6)
2022	Women	101	44	43.6 (34.3-53.3)	41.4 (32.5-50.4)
2022	Men	132	36	27.3 (20.4-35.4)	28.8 (21.5-36.1)
2023	Women	102	35	34.3 (25.8-43.9)	31.3 (22.5-40.1)
2023	Men	136	33	24.3 (17.8-32.1)	27.1 (19.6-34.6)
2024	Women	95	30	31.6 (23.1-41.5)	28.8 (20.3-37.2)
2024	Men	122	19	15.6 (10.2-23.0)	18.6 (11.3-25.9)
2025	Women	138	51	37.0 (29.4-45.3)	34.3 (26.7-41.9)
2025	Men	183	56	30.6 (24.4-37.6)	32.2 (25.6-38.8)

WHAT THIS MEANS

Heart case fatality in 2025 was higher than Stroke case fatality and remained roughly one in three events. The crude estimate was higher for women than men, but this gap narrowed after age adjustment and the uncertainty ranges overlap. Heart estimates vary more from year to year than Stroke estimates, partly reflecting smaller event numbers; decisions should therefore consider the multi-year pattern rather than one annual point.

STROKE EVENTS

Thirty-day case fatality after Stroke events

Annual results by sex.

CRUDE | WOMEN + MEN | 2025

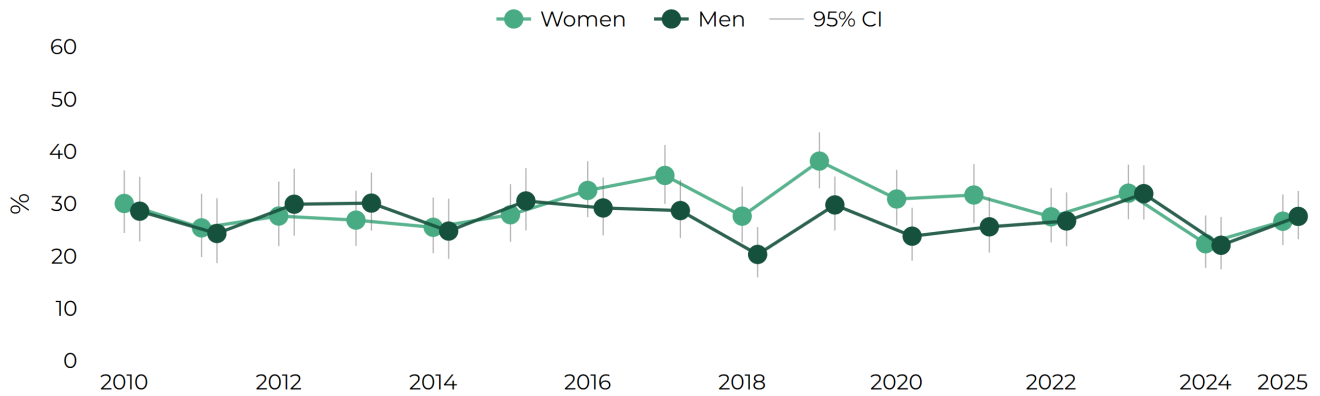
27.1%

95% CI 23.9-30.6

AGE-ADJUSTED | WOMEN + MEN | 2025

27.7%

95% CI 24.4-31.0



Recent results by sex

Year	Sex	Events	Deaths	Crude % (95% CI)	Adjusted % (95% CI)
2021	Women	259	82	31.7 (26.3-37.6)	29.9 (24.5-35.4)
2021	Men	258	66	25.6 (20.6-31.2)	27.5 (22.2-32.8)
2022	Women	280	77	27.5 (22.6-33.0)	26.1 (21.2-31.1)
2022	Men	281	75	26.7 (21.9-32.2)	29.0 (23.9-34.2)
2023	Women	303	97	32.0 (27.0-37.5)	30.2 (25.2-35.1)
2023	Men	307	98	31.9 (27.0-37.3)	33.9 (28.8-39.0)
2024	Women	264	59	22.3 (17.7-27.7)	22.7 (17.7-27.7)
2024	Men	263	58	22.1 (17.5-27.4)	24.5 (19.2-29.8)
2025	Women	319	85	26.6 (22.1-31.8)	25.5 (20.9-30.1)
2025	Men	359	99	27.6 (23.2-32.4)	29.9 (25.1-34.6)

WHAT THIS MEANS

In 2025, crude stroke case fatality was almost the same for women and men. Age adjustment produced a somewhat higher estimate for men, but the uncertainty ranges overlap. The combined estimate was below 2023 and within the broad range seen across the series. The annual pattern does not show a consistent year-on-year improvement. Continued monitoring remains important.

METHODS

How this report was produced

The same steps were used for every year and for women, men and both sexes combined.

How case fatality is calculated

$$\text{Case fatality within 30 days (\%)} = \frac{\text{deaths within 30 days}}{\text{eligible index events}} \times 100$$

What was counted?	BNR hospital records of Heart and Stroke events from 2010 to 2025. All CVD combines the Heart and Stroke event groups.
Which event was used?	Within each calendar year, we used each person's first Heart event and first Stroke event. Later events of the same type during that year were not counted. A person who experienced both types could contribute once to the Heart series and once to the Stroke series, and therefore twice to All CVD. All CVD is consequently an event total, not a count of distinct people or deaths.
What counted as a death?	We counted a death from any cause occurring on the event date or during the following 30 days. The recorded cause of death did not have to be cardiovascular. A death was counted when it was reported in the hospital record or when the event record linked to a death in the all-deaths register.
How were records linked?	Hospital and death records were linked using predefined exact matching rules. We first used a valid national registration number when it identified one person and there was no conflicting information. When this was not possible, we required exact agreement on specified combinations of name, sex and date of birth, with a limited age-in-years fallback. A possible match was accepted only when it identified one unique person; ambiguous matches remained unlinked.
What does crude mean?	The crude result is the percentage actually observed in that year's event group: eligible index events followed by death within 30 days, divided by all eligible index events, multiplied by 100. It makes no allowance for differences in patients' ages. It describes the experience recorded by BNR in that year and is the main result in this report. It is a percentage among recorded events, not a population mortality rate.
Why adjust for age?	The chance of dying within 30 days generally increases with age. A year with older event patients can therefore have a higher crude percentage even if outcomes at the same ages have not worsened. The age-adjusted result estimates what each year's percentage would be if its age mix matched a fixed Barbados reference group. The All-CVD, Heart and Stroke series each use their own pooled 2010-2025 reference group. Adjusted results can therefore be compared over time within the same series, but not directly between the three series. We calculate them using logistic regression and predictive margins.
What does the 95% CI show?	The 95% confidence interval shows the statistical precision of the estimated percentage. Wider intervals indicate greater uncertainty, usually because fewer events were available. Crude intervals use the Wilson method; age-adjusted intervals are calculated from the statistical model and allow for repeat records from the same person. These intervals describe uncertainty arising from the observed numbers; they do not account for missed events, incomplete records or deaths that were not successfully linked.
How was privacy protected?	Counts from 1 to 5 were suppressed. We also suppressed additional values when necessary to prevent a protected count from being calculated by subtracting other published values. Zero counts were not automatically suppressed. Disclosure checks were applied to the complete proposed public dataset, and every result displayed in this report passed those checks.

Data used

CVD event release: January 2026. All-deaths release: 2026-07. Death follow-up runs through 30 January 2026, giving every event through 31 December 2025 a complete 30-day follow-up period.